MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;DEFINING MODERATE ACUTE MALNUTRITION IN INDIAN CLINICS;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;DEFINING MODERATE ACUTE MALNUTRITION IN INDIAN CLINICS;CLINIC-READY DIAGNOSTIC CRITERIA AND MISCLASSIFICATION PITFALLS;

Moderate acute malnutrition (##MAM##) refers to acute wasting without nutritional oedema. For children 6–59 months, ##WHO## defines moderate wasting as weight-for-height/length (##WHZ##/##WLZ##) between −2 and −3 z-scores and/or ##MUAC## 115–124 mm, with no bilateral pitting oedema. In Indian clinics, a safety-first operational definition is: moderate wasting by either metric, clinically stable with appetite, and no acute danger signs or complications requiring inpatient care. Document the metric used, because program eligibility may depend on it. Do not equate “underweight” (low weight-for-age) with ##MAM##; underweight may reflect chronic stunting. Check oedema correctly by pressing both feet for several seconds and looking for persistent pitting. Repeat any borderline measurement and triangulate with the clinical picture (recent weight loss, poor intake, frequent infection). Explicitly label the child as uncomplicated ##MAM## versus wasting with complications, because the follow-up intensity and escalation threshold differ. Indian facility-based malnutrition materials often describe ##MAM## as 70–80% of median weight-for-height (or z-score −3 to −2) or ##MUAC## 115–125 mm with no oedema, managed as an outpatient child who is clinically well and has appetite when supplementary feeding is available. Some manuals include unit typos; interpret arm circumference values in millimetres to stay aligned with ##WHO## thresholds. A practical bedside approach is a one-minute wasting screen at every sick-child visit: look for visible wasting, check oedema, measure ##MUAC## (or weight/length if feasible), and ask about appetite and recent weight loss. This reduces late recognition after progression to ##SAM## or after complications appear.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- National Health Mission. Operational guidelines on Facility Based Management of Children with SAM (FBMC) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/operational_guidelines_on_fbmc_with_sam.pdf

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Aayog.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;BURDEN AND DRIVERS OF ACUTE WASTING IN INDIA;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;BURDEN AND DRIVERS OF ACUTE WASTING IN INDIA;POPULATION BURDEN AND CLINICALLY ACTIONABLE DETERMINANTS;

At a population level, wasting remains common in India. The ##NFHS-5## (2019–2021) fact sheet reports 19.3% of under-5 children are wasted (weight-for-height < −2) and 7.7% are severely wasted (< −3). “Moderate wasting” is not listed as a separate key indicator, but can be approximated as wasted minus severely wasted, implying about 11.6% moderately wasted by subtraction. Clinically, this matters because most wasting presents in the moderate category where effective outpatient care should prevent progression and avoidable deaths. Determinants in Indian guidance consistently reflect a convergence of inadequate diet quality or quantity, inappropriate feeding practices, recurrent infections (especially diarrhoea and respiratory infections), and suboptimal ##WASH##. The relationship is bidirectional: infection reduces appetite and increases catabolism, while undernutrition impairs immune responses and increases illness severity. In clinic, avoid attributing wasting to “poverty only.” When you diagnose ##MAM##, write down two likely drivers for that child—one dietary (e.g., low energy density, inadequate meal frequency, milk displacement) and one health-related (e.g., recurrent diarrhoea, suspected ##TB##, chronic cough, anaemia). Your plan should address both in parallel: nutrition rehabilitation plus active diagnosis and treatment of illness. Programmatic compendia from ##NITI## Aayog and ##UNICEF## emphasize that wasting is sensitive to short-term shocks (food insecurity, seasonal infection peaks) and requires integrated prevention and treatment across community and facility systems, with explicit attention to both ##MAM## and ##SAM##. A practical implication is to use acute wasting as a “sentinel event”: screen for food access and caregiving constraints, but also for treatable disease when symptoms are persistent or atypical. Generic diet charts without addressing the illness– malnutrition cycle and caregiver constraints rarely change outcomes.

References:

- Ministry of Health and Family Welfare. NFHS-5 (2019–21) National fact sheet — https://mohfw.gov.in/sites/default/files/NFHS-5_Phase-II_0.pdf

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Aayog.pdf

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- PLOS ONE. Wasting and stunting as risk factors for mortality in children under 5 years (pooled analysis) — https://journals.plos.org/plosone/article?id=10.1371%2Fjournal.pone.0064636

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;PATHOPHYSIOLOGY OF WASTING AND CLINICAL CORRELATES;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;PATHOPHYSIOLOGY OF WASTING AND CLINICAL CORRELATES;INFECTION CATABOLISM AND GUT DYSFUNCTION IN WASTING;

Acute wasting reflects a mismatch between intake or absorption and physiological needs, amplified by the infection–nutrition cycle. Intercurrent infections increase energy expenditure and inflammatory catabolism, while anorexia and feeding disruption reduce intake; micronutrient deficits further impair immune function, increasing infection recurrence. In clinical terms, persistent poor appetite is a marker of unresolved illness or inflammation rather than “behaviour,” and should trigger morbidity screening and closer follow-up.

- In low-resource settings, gut dysfunction contributes to blunted recovery. Environmental enteric dysfunction (##EED##) is a subclinical enteropathy linked to repeated enteric pathogen exposure; it causes intestinal inflammation, impaired absorption and barrier disruption. Children may present with fluctuating appetite, frequent minor infections, and limited weight gain despite apparently adequate calories. Counselling must therefore include ##WASH## actions and prompt treatment of diarrhoea to reduce ongoing gut injury.

- Recovery requires a planned “top-up” over usual needs. For children 6–59 months with moderate wasting, the 2023 ##WHO## guideline estimates total daily energy around 100–130 kcal/kg/day during recovery; supplements (##THR##/##HCM## or ##RUSF## where available) should add to, not replace, family food. Relapse within weeks of discharge often reflects unresolved infection burden, food insecurity, or caregiving constraints; treat relapse as an indication to re-open the driver assessment and re-intensify follow-up rather than repeating diet advice alone.

References:

- Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- Immune dysfunction as a cause and consequence of malnutrition — https://pmc.ncbi.nlm.nih.gov/articles/PMC4889773/

- Environmental enteric dysfunction: an overview — https://pmc.ncbi.nlm.nih.gov/articles/PMC4472379/

- The interaction between nutrition and infection — https://academic.oup.com/cid/article/46/10/1582/294025

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;TRIAGE AND DISPOSITION FOR ACUTE WASTING;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;TRIAGE AND DISPOSITION FOR ACUTE WASTING;EMERGENCY AND DANGER SIGNS GUIDING ADMISSION OR REFERRAL;

Disposition in suspected ##MAM## starts with triage, not anthropometry. In a busy ##OPD## or emergency room, first confirm acute wasting criteria and record both ##MUAC## and ##WHZ##/##WLZ## when feasible, because program entry metrics vary across India. Then apply facility-based paediatric triage logic (##ETAT##). Emergency signs requiring immediate emergency treatment and usually admission or urgent referral include: not breathing or gasping; obstructed breathing; central cyanosis; severe respiratory distress (grunting, head nodding, severe lower chest indrawing, apnoeic spells, stridor in a calm child, or inability to feed due to respiratory distress); shock (cold hands with ##CRT## >3 seconds plus weak and fast pulse); coma on ##AVPU## or ongoing convulsions; and severe dehydration in diarrhoea (diarrhoea plus at least two of lethargy, sunken eyes, very slow skin pinch). If any emergency sign is present, stabilize first and do not label the child “uncomplicated.” Also screen for general danger signs used in ##IMNCI##/##IMCI##: inability to drink or breastfeed, vomiting everything, convulsions, and lethargy or unconsciousness. The presence of any emergency or danger sign, or complications such as severe pneumonia, temperature instability (fever ≥39°C or hypothermia <35°C), suspected hypoglycaemia, severe palmar pallor, extensive infection requiring ##IM## antibiotics, or caregiver inability to provide safe home care, should trigger admission or referral as “complicated wasting.”

References:

- National Health Mission. Strengthening facility based paediatric care operational guidelines (ETAT triage) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/Strenghtening_Facility_Based_Paediatric_Care-Operational_Guidelines.pdf

- National Health Mission. Integrated management of neonatal and childhood illness guidelines (2 months to 5 years) — https://www.nhm.gov.in/images/pdf/guidelines/nrhmguidelines/stg/imnci-2-months-to-5-years.pdf

- National Health Mission. Operational guidelines on Facility Based Management of Children with SAM (FBMC) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/operational_guidelines_on_fbmc_with_sam.pdf

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;TRIAGE AND DISPOSITION FOR ACUTE WASTING;HYDRATION ASSESSMENT AND ORAL REHYDRATION PLANS;

Fluid management is a frequent error point when “malnutrition” is treated as a single category. For children 6–59 months with moderate wasting (##MAM##) and dehydration but not shock, the 2023 ##WHO## wasting guideline recommends low-osmolarity ##ORS## in accordance with standard diarrhoea recommendations for all children, reserving the malnutrition-specific formulation ##ReSoMal## for severe wasting and/or nutritional oedema. In practical terms, a child who truly meets ##MAM## criteria (no oedema and not severely wasted) should receive the usual dehydration plans, but with a lower threshold for observation or admission when intake is poor or the caregiver cannot ensure reliable home care. A clinic-ready algorithm is: if there is no dehydration, continue feeding, give extra fluids after each loose stool, and arrange early review because acute illness can precipitate deterioration. If there is some dehydration (restless or irritable, sunken eyes, drinks eagerly, skin pinch slow), manage with oral rehydration therapy under observation when feasible, reassess frequently, and confirm that feeding resumes as soon as tolerated. If severe dehydration is present (diarrhoea plus at least two of lethargy, sunken eyes, very slow skin pinch) or any shock sign exists, treat emergently per standard protocols and admit or urgently refer. Residents should be taught a single operational rule: in ##MAM##, treat dehydration like in any child unless the child has severe wasting or oedema, but never allow the nutrition label to delay emergency stabilization or disposition decisions.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- National Health Mission. Integrated management of neonatal and childhood illness guidelines (2 months to 5 years) — https://www.nhm.gov.in/images/pdf/guidelines/nrhmguidelines/stg/imnci-2-months-to-5-years.pdf

- National Health Mission. Strengthening facility based paediatric care operational guidelines (ETAT triage) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/Strenghtening_Facility_Based_Paediatric_Care-Operational_Guidelines.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;ANTHROPOMETRY AND BORDERLINE INTERPRETATION IN OUTPATIENT CARE;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;ANTHROPOMETRY AND BORDERLINE INTERPRETATION IN OUTPATIENT CARE;MEASUREMENT TECHNIQUE FOR MUAC LENGTH AND OEDEMA;

Anthropometry is a clinical skill because it determines diagnosis, risk stratification, entitlement to supplementary rations, and follow-up intensity. For ##MUAC##, use a non-stretch tape on the left arm: identify the midpoint between acromion and olecranon, keep the arm relaxed, wrap the tape snugly without compressing soft tissue, read to the nearest millimetre, and repeat at least twice. For weight and length/height, ensure the scale and length board are functional and calibrated, remove

heavy clothing, and confirm age and measurement position. Oedema assessment must be systematic. Check bilateral pitting oedema by pressing both feet for several seconds and confirming persistent pitting. Unilateral swelling suggests local pathology and should not be labelled as nutritional oedema. Because small measurement errors can move a child across ##MAM## thresholds, repeat any borderline value and document the final confirmed measurement, including the tool used. In India, local program entry criteria may differ from clinic definitions (for example, some community workflows emphasize ##WHZ## plus oedema). To reduce exclusion due to administrative ambiguity, write both ##MUAC## and ##WHZ##/##WLZ## in the record when feasible, and note whether measurements were repeated. Standardizing tapes and technique within the clinic reduces systematic misclassification and prevents inappropriate discharge or missed escalation. Where measurement quality is uncertain, err on the side of safety: treat a high-risk child with borderline values as needing intensified counselling, closer follow-up, and strong program linkage, rather than downgrading risk because of a single imprecise reading.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- National Health Mission. Operational guidelines on Facility Based Management of Children with SAM (FBMC) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/operational_guidelines_on_fbmc_with_sam.pdf

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Aayog.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;ANTHROPOMETRY AND BORDERLINE INTERPRETATION IN OUTPATIENT CARE;MANAGEMENT OF BORDERLINE VALUES AND DISCORDANT METRICS;

Borderline values (for example ##MUAC## 124–125 mm or ##WHZ## near −2) and discordance between ##MUAC## and ##WHZ## are common. Discordance is expected because the two measures identify overlapping but not identical risk phenotypes, and mortality risk is highest when both ##MUAC## and ##WHZ## are low. The first response to discordant classification is technical: repeat measurements, confirm equipment calibration, ensure correct length/height technique, and measure when the child is calm. After technical confirmation, use the more severe indicator for clinical risk stratification and follow-up intensity. For program eligibility, use the locally required entry metric, but explicitly document discordance so clinical risk is not underestimated when the child is “not eligible” by the program’s preferred measure. ##WHO## provides practical risk factors to decide when counselling alone is unlikely to be sufficient and when children should be prioritized for more intensive dietary support and closer follow-up: ##MUAC## 115–119 mm, weight-for-age (##WAZ##) < −3, age <24 months, failure to recover after other interventions, relapse or prior severe wasting, comorbidity (for example ##HIV##, ##TB##, disability), and severe adverse social circumstances. In

clinics without access to specially formulated foods, treat these risk factors as triggers for tighter follow-up, lower threshold to investigate, and stronger linkage to supplementary feeding entitlements. When uncertainty persists, the safe approach is to manage “as if eligible” for intensified support: schedule the next visit earlier, check intake and morbidity in detail, and ensure that the family can actually access and use the supplement without displacement or sharing.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- PLOS ONE. MUAC and WHZ as predictors of mortality and programme outcomes in children under 5 years — https://journals.plos.org/plosone/article?id=10.1371%2Fjournal.pone.0219745

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Aayog.pdf

- PLOS ONE. Wasting and stunting as risk factors for mortality in children under 5 years (pooled analysis) — https://journals.plos.org/plosone/article?id=10.1371%2Fjournal.pone.0064636

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;CLINICAL SYNDROME RECOGNITION AND BEDSIDE ASSESSMENT IN MODERATE WASTING;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;CLINICAL SYNDROME RECOGNITION AND BEDSIDE ASSESSMENT IN MODERATE WASTING;STRUCTURED OUTPATIENT HISTORY EXAMINATION AND FEEDING OBSERVATION;

Moderate wasting should be assessed as a clinical syndrome, not only an anthropometry label. In the first minutes of an ##OPD## visit, confirm three disposition drivers: current appetite, caregiver report of recent weight loss or falling ##MUAC##, and any emergency or general danger signs (##ETAT##/##IMNCI##). If the child is stable, extend the history to feeding function: who feeds, meal duration (prolonged meals suggest fatigue, oral-motor problems, coercive feeding, or poor technique), refusal patterns, vomiting, and cough or choking with feeds. Screen for illness drivers of catabolism (diarrhoea, fever, cough or fast breathing, ear discharge, skin infection) and for contextual constraints (food insecurity, ration access, caregiver workload, maternal illness or depression).

- Examination should “front-load” early complications that change follow-up intensity: hydration status, respiratory rate and work of breathing, temperature, pallor, and bilateral pitting oedema. Document visible loss of subcutaneous fat or reduced mid-arm muscle bulk as a baseline. Look specifically for micronutrient deficiency patterns (periorificial dermatitis, angular cheilitis, glossitis, xerosis or Bitot spots) and for rickets features that impair feeding endurance (swollen wrists, rachitic

rosary, delayed milestones). Abdominal distension or hepatosplenomegaly should prompt evaluation for chronic infection, ##TB##, or malabsorption.

- At least once at baseline, observe a real feed or a simulated meal. A short observation detects unsafe swallowing, inappropriate textures, and poor caregiver-child interaction that 24-hour recall misses, and it creates a concrete “one change” plan for the next visit.

References:

- Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- IMNCI Chart Booklet for Medical Officers (2023) — https://nhm.gov.in/images/pdf/programmes/child-health/guidelines/IMNCI-Module-2023-For-Medical-Officers/IMNCI-Chart-booklet-Medical-Officer-2023.pdf

- Strengthening facility-based paediatric care: operational guidelines — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/Strenghtening_Facility_Based_Paediatric_Care-Operational_Guidelines.pdf

- Enhancing optimal infant and young child feeding practices: operational guidelines — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/Enhancing-optimalIYCFpractices.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;COUNSELLING AND HOME DIET THERAPY IN UNCOMPLICATED MODERATE WASTING;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;COUNSELLING AND HOME DIET THERAPY IN UNCOMPLICATED MODERATE WASTING;THERAPEUTIC GOALS AND INDICATIONS FOR SUPPLEMENTATION;

In uncomplicated ##MAM##, the primary therapeutic objective is recovery from wasting while preventing progression to ##SAM##, supporting neurodevelopment, and reducing infection recurrence. Counselling is not an “add-on”; it is the core intervention because ##WHO## anchors moderate wasting care in a nutrient-dense diet plus identification and treatment of medical and psychosocial drivers. Supplement selection should be risk-stratified rather than automatic. In routine (non-humanitarian) settings, the most current ##WHO## recommendations prioritize counselling and nutrient-dense home diets first, and then selectively recommend specially formulated foods for children at higher risk of poor outcomes (for example ##MUAC## 115–119 mm, ##WAZ## < −3, age <24 months, relapse, comorbidity, disability, or severe social adversity). In high-risk humanitarian contexts, supplementation may be considered for all children 6–59 months with moderate wasting alongside counselling and household food support. When a specially formulated supplement is used, ##WHO## conditionally prefers lipid-based nutrient supplements (##LNS##). If ##LNS## is not available, improved fortified blended foods are preferred over unimproved blends. The supplement

is intended to contribute about 40–60% of total daily energy needs during recovery, with total energy needs often estimated around 100– 130 kcal/kg/day, so supplementation should be planned as a top-up that does not displace family foods. In Indian outpatient practice, the practical question is often not “which product,” but “can the family reliably access and use a supplement,” so eligibility decisions should be coupled with a concrete linkage plan and a follow -up schedule that can detect displacement, sharing, or clinical deterioration early.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- PLOS ONE. Wasting and stunting as risk factors for mortality in children under 5 years (pooled analysis) — https://journals.plos.org/plosone/article?id=10.1371%2Fjournal.pone.0064636

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

- National Health Mission. Guidelines for enhancing optimal infant and young child feeding practices — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/EnhancingoptimalIYCF-practices.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;COUNSELLING AND HOME DIET THERAPY IN UNCOMPLICATED MODERATE WASTING;QUANTIFIED MEAL FREQUENCY ENERGY DENSITY AND PROTEIN ANCHORS;

Caregivers follow numbers better than general diet charts. Frame recovery as a quantified “topup” over usual intake: for children 6 –59 months with moderate wasting, total daily energy during recovery is often around 100–130 kcal/kg/day in ##WHO## guidance. The outpatient prescription should therefore specify how the family will increase energy density and protein without relying on large portions when appetite is limited.

- A clinic-ready method is a three-number prescription written in the notes: (1) meals and snacks per day (aim for at least 5–6 feeds per day in ages 6–24 months, and “three meals plus two snacks” in ages 2–5 years); (2) protein anchors per day (aim for 2–3 protein touches daily using affordable local options such as dal, chana, rajma, egg, milk, curd, or soy, aligned with age-appropriate protein targets in ##ICMR##-##NIN## summaries); and (3) energy density upgrade (add oil, ghee, groundnut, or til paste to each main meal after cooking). Emphasize thick consistency (food should stay on a spoon) and avoid low-energy liquids (tea, thin gruels) that suppress appetite. If milk is used, counsel against dilution and against milk replacing solids.

- When a supplementary ration is provided, prescribe it as “in addition to” family food and time it as a snack or after meals to reduce displacement.

- Non-response is often non-intake. At each follow-up, verify actual intake with a 24-hour recall plus portion estimation using household measures (katori, spoon, roti count): what was offered versus finished, who fed the child, how long meals took, and whether any supplement was shared or diluted. Use this audit to agree on one feasible change for the next 14 days.

References:

- Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- Nutrient requirements and recommended dietary allowances for Indians: brief note (ICMR-NIN) — https://www.nin.res.in/rdabook/brief_note.pdf

- Enhancing optimal infant and young child feeding practices: operational guidelines — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/EnhancingoptimalIYCFpractices.pdf

- Improving Young Children’s Diets During the Complementary Feeding Period: UNICEF Programming Guidance (2020) — https://www.unicef.org/media/93981/file/ComplementaryFeeding-Guidance-2020.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;COUNSELLING AND HOME DIET THERAPY IN UNCOMPLICATED MODERATE WASTING;AGE-BANDED FEEDING PRESCRIPTION FOR INFANTS 6 TO 11 MONTHS;

For infants 6–11 months with ##MAM##, feeding rehabilitation must be tightly structured because small deficits in meal frequency and energy density rapidly translate into weight loss. Use national ##IYCF## guidance as a backbone, but convert it into a “tomorrow plan” the caregiver can execute. Ages 6–8 months: continue breastfeeding on demand. Offer 2–3 thick meals per day; add 1 snack if appetite allows. Start with well-mashed foods and increase quantity from a few spoonfuls toward about half of a 250 mL cup over days. Emphasize thickness (food should stay on a spoon) and energy density (mix 1 teaspoon oil or ghee into khichdi or porridge after cooking). Ages 9–11 months: continue breastfeeding. Offer 3–4 meals plus 1–2 snacks daily. Foods can be mashed, finely chopped, or safe finger foods, aiming for around half of a 250 mL bowl per meal with progressive increase. Practical Indian options include thick khichdi (rice plus dal) with oil or ghee, soft roti pieces soaked in thick dal, curd or fruit as a snack if tolerated, and egg or extra dal as a protein add-on when culturally acceptable. At every visit, agree on one change the caregiver will do daily for the next 14 days (for example, add one snack, stop dilution of milk, or add oil to one main meal). Smaller sustained changes outperform complex diet charts. Coach responsive feeding: the caregiver should sit with the infant, offer patiently without force, and give meals at predictable times rather than continuous grazing. Avoid low-energy liquids (tea, thin gruels, watery “health drinks”) that suppress appetite without providing adequate calories. If complementary feeding triggers caregiver fear after

diarrhoea, explain that illness often causes diarrhoea, and feeding should continue with appropriate rehydration and zinc when indicated.

References:

- National Health Mission. Guidelines for enhancing optimal infant and young child feeding practices — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/EnhancingoptimalIYCF-practices.pdf

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- World Health Organization. Zinc supplementation in diarrhoea (eLENA) — https://www.who.int/tools/elena/interventions/zinc-diarrhoea

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;COUNSELLING AND HOME DIET THERAPY IN UNCOMPLICATED MODERATE WASTING;FEEDING PRESCRIPTION FOR CHILDREN 12 TO 59 MONTHS;

For children 12–59 months with ##MAM##, the counselling prescription should specify meal frequency, energy density, and a realistic protein source, while accounting for time, cost, and caregiver capacity. Ages 12–23 months: continue breastfeeding if ongoing. Prescribe 3–4 meals plus 1–2 snacks daily. Portions are typically three-quarters to one 250 mL cup per meal depending on energy density. Use “energy add-ons” rather than larger portions: add a spoon of oil or ghee to one or two main meals, and add a daily protein anchor such as egg or an extra dal or legume portion. If milk is used, counsel that dilution reduces calorie and protein density; use safe full-strength milk, but do not allow milk to replace solids. Ages 24–59 months: prescribe family foods as 3 main meals plus 2 snacks (“three-plus-two”). Maintain energy density by adding oil or ghee to each main meal, and avoid watery foods that displace calories. Replace low-nutrient grazing (tea, biscuits, packaged snacks) with planned snacks (banana, roasted chana, curd, egg, or a legume-based snack). Document a minimum counselling dataset in the note: problem statement; main barrier; the single agreed “one change” for 14 days; a meal and snack count; two specific foods the caregiver will give daily; and one monitoring question for the next visit (for example, “in the last 3 days, on how many days did the child finish the extra snack?”). When a supplementary ration is provided, prescribe its timing in relation to meals (preferably after a main meal) to reduce displacement of home foods, and ask the caregiver to describe exactly how the child is fed. This shifts counselling from advice to an executable plan.

References:

- National Health Mission. Guidelines for enhancing optimal infant and young child feeding practices — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/EnhancingoptimalIYCF-practices.pdf

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- Ministry of Women and Child Development. Mission Saksham Anganwadi and Poshan 2.0 scheme guideline — https://wcd.delhi.gov.in/sites/default/files/WCD/generic_multiple_files/final_saksham_anganwadi_and_mission.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;COUNSELLING AND HOME DIET THERAPY IN UNCOMPLICATED MODERATE WASTING;MICRONUTRIENT SUPPLEMENTATION AND INFECTION PREVENTION LINKAGES;

Adjunct interventions in ##MAM## should be integrated with India's routine platforms rather than relying on costly private "nutrition tonics." Micronutrient gaps and infection burdens often coexist, so outpatient plans should routinely check that the child is receiving age-appropriate prophylaxis and treatment through public programmes. Anaemia prevention: under Anaemia Mukt Bharat (##AMB##), prophylactic iron –folic acid (##IFA##) is provided as ##IFA## syrup for ages 6–59 months at 1 mL twice weekly (each 1 mL contains 20 mg elemental iron and 100 mcg folic acid). For ages 5 – 10 years, th e programme provides a weekly tablet containing 45 mg iron and 400 mcg folic acid, and for ages 10–19 years a weekly tablet containing 60 mg iron and 500 mcg folic acid. In children with clinical anaemia, treat according to severity and national anaemia pa thways rather than continuing prophylaxis alone. Deworming: under National Deworming Day guidance, albendazole is given as a single 200 mg dose for ages 1–2 years and 400 mg for ages 2–19 years, with dosing delivered through campaign platforms and frontline workers. Vitamin A prophylaxis is delivered between 9 months and 5 years (100,000 ##IU## at 9 months, then 200,000 ##IU## every 6 months up to 5 years, per programme schedule). For acute diarrhoea, continue feeding, give low-osmolarity ##ORS## for rehydration, and provide zinc per ##WHO## guidance (20 mg daily for 10–14 days, or 10 mg daily if under 6 months). Use each ##MAM## visit to verify immunization status and reinforce simple infection-prevention measures (handwashing, safe water, prompt treatment-seeking), because reducing infection recurrence is often as important as increasing calories for sustained recovery.

References:

- National Health Mission. Anaemia Mukt Bharat: prophylactic IFA supplementation schedule — https://nhm.gov.in/index1.php?lang=1&level=3&lid=797&sublinkid=1448

- National Health Mission. National Deworming Day: FAQ for frontline workers — https://nhm.gov.in/images/pdf/NDD/FAQ/FAQ_for_NDD-FrontlineWorkers_Eng.pdf

- National Health Mission. Vitamin A supplementation programme — https://nhm.gov.in/index1.php?lang=1&level=3&lid=801&sublinkid=1452

- World Health Organization. Zinc supplementation in diarrhoea (eLENA) — https://www.who.int/tools/elena/interventions/zinc-diarrhoea

- National Health Mission. Integrated management of neonatal and childhood illness guidelines (2 months to 5 years) — https://www.nhm.gov.in/images/pdf/guidelines/nrhmguidelines/stg/imnci-2-months-to-5-years.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;COUNSELLING AND HOME DIET THERAPY IN UNCOMPLICATED MODERATE WASTING;CATCH-UP IMMUNIZATION REFERRAL WORKFLOW DURING WASTING FOLLOW-UP;

Every ##MAM## visit is an opportunity to close immunization gaps that perpetuate infectiondriven weight loss. Make immunization status a documented field at day 0 and again at the week 4 response node. If the caregiver has no card, treat the child as potentially underimmunized until records are verified through the nearest routine immunization session (##ANM##/##ASHA## or facility register). A clinic-ready workflow is: (1) check the child's age and vaccines already received; (2) identify what is due under the National Immunization Schedule; (3) give the caregiver a written note that states "acute wasting: prioritize routine immunization" and the nearest session location and date; and (4) verify completion at the next follow-up. Vaccination is recommended in clinically stable children with uncomplicated moderate wasting; do not delay simply because the child is labelled ##MAM##. Emphasize that vaccine series do not need restarting; missed doses are given as catch-up, following schedule guidance at the immunization site. Mild intercurrent illness is usually not a contraindication, so waiting for "perfect health" delays protection. For migrant families and children who repeatedly miss sessions, link proactively to special drives such as Mission Indradhanush and to community platforms (##VHSND##). Document the linkage the same way you document supplementary feeding: where the family will go, when, and what will be checked at the next visit. This converts immunization from generic advice into an executable plan that reduces future diarrhoea and pneumonia episodes during recovery.

References:

- National Immunization Schedule (India, English) — https://resources.risemohfw.in/uploads/2022/12/30/nis-english.pdf

- Mission Indradhanush Guidelines (NHM) — https://www.nhm.gov.in/New_Updates_2018/NHM_Components/Immunization/Guildelines_for_immunization/Mission_Indradhanush_Guidelines.pdf

- IMNCI: standard treatment guidelines for 2 months to 5 years — https://www.nhm.gov.in/images/pdf/guidelines/nrhm-guidelines/stg/imnci-2-monthsto5years.pdf

- Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;SUPPLEMENTARY FEEDING REGIMENS AND SAFE PRODUCT USE;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;SUPPLEMENTARY FEEDING REGIMENS AND SAFE PRODUCT USE;USING ICDS ENTITLEMENTS AND COUNSELLING ON NUTRITION NORMS;

In India, the most consistently available supplementary feeding platform for children with wasting is the supplementary nutrition programme delivered through anganwadi services under ##ICDS## and Mission Poshan. Scheme guidance frames supplementary nutrition as bridging the gap between recommended dietary allowances and average intake, delivered for a minimum of about 300 days per year through the anganwadi network, usually as take-home rations (##THR##) and/or hot cooked meals (##HCM##), with registration-linked entitlements. Nutrition norms provide a clinical counselling anchor. Mission Poshan guidance lists daily norms for children 6–72 months of about 500 kcal and 12–15 g protein, with higher norms of about 800 kcal and 20–25 g protein for children classified as severely malnourished (noting that norms may be under revision). The 2017 supplementary nutrition rules under ##ICDS## also describe enhanced provisions for malnourished children, including energy and protein targets for ##THR## in younger children and added calories and protein in older preschool children on top of the standard meal. These figures help you set expectations: the ration is a supplement, so home foods must still provide substantial energy and protein, and lack of improvement may reflect inconsistent receipt, sharing, or preparation problems rather than a medical failure. Link the programme ration to the ##WHO## concept of a top-up. If the child is high-risk and a supplemental food is being used, aim for a supplementary energy contribution of roughly 40– 60% of total daily energy needs during recovery, while actively protecting family-food intake so the supplement accelerates recovery rather than replacing meals.

References:

- Ministry of Women and Child Development. Mission Saksham Anganwadi and Poshan 2.0 scheme guideline — https://wcd.delhi.gov.in/sites/default/files/WCD/generic_multiple_files/final_saksham_anganwadi_and_mission.pdf

- Government of India. 2017. Supplementary Nutrition Rules under ICDS — https://hpfoodcommission.nic.in/pdf/Supplementary_Nutrition%20_ICDS_%20Rules_2017.pdf

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;SUPPLEMENTARY FEEDING REGIMENS AND SAFE PRODUCT USE;HOUSEHOLD-LEVEL PRESCRIBING AND ADHERENCE AUDIT FOR THR AND HOT COOKED MEALS;

##THR## and hot cooked meals (##HCM##) work only when clinicians prescribe their use operationally, like a medication, rather than assuming that “receipt equals intake.” Start by confirming the supply reality: the last pickup date, the number of days covered, and any stockouts. If the family cannot attend daily ##HCM## because of distance or caregiver work, shift emphasis to predictable ##THR## access and intensify the home diet prescription so the child still receives an energy-dense plan.

- Make the child’s share explicit. Tell the caregiver: “This ration is the child’s medicine.” Agree on a storage and serving plan that reduces sharing (separate container, fixed serving time, caregiver supervision) and document the agreement. Demonstrate one preparation method with the family’s utensils and specify the goal: thick consistency with an added fat source to increase energy density. Warn against over-dilution into watery gruel or tea, which reduces calories and increases contamination risk.

- Prevent displacement by prescribing timing: use ##THR## as an extra snack or after meals, not as a replacement for lunch or dinner. At follow-up, perform a brief adherence audit: days received, days consumed by the index child, sharing, storage, and preparation errors.

- When programme problems occur (non-receipt, poor-quality packets, repeated stockouts), use an escalation ladder that families can act on: document the issue with dates and quantities; provide a written linkage note to the ##AWC## supervisor/##CDPO## stating clinical risk and the need for uninterrupted supplementary nutrition; and involve facility social support services when persistent failures threaten recovery.

References:

- Supplementary Nutrition (ICDS) Rules, 2017 — https://hpfoodcommission.nic.in/pdf/Supplementary_Nutrition%20_ICDS_%20Rules_2017.pdf

- Saksham Anganwadi and Mission Poshan 2.0: guidelines — https://wcd.delhi.gov.in/sites/default/files/WCD/generic_multiple_files/final_saksham_anganwadi_and_mission.pdf

- Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- Community-based management of acute malnutrition in India: a report (NITI Aayog and UNICEF) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Aayog.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;SUPPLEMENTARY FEEDING REGIMENS AND SAFE PRODUCT USE;PRESCRIBABLE DOSING AND SAFE USE OF RUSF AND LNS;

When specially formulated foods are available, prescribe them like a medicine: name, dose, duration, and a plan to monitor use. From the ##WHO## recovery target of about 100–130 kcal/kg/day total energy needs and a supplement contribution of about 40–60%, the supplementary component is roughly 40–78 kcal/kg/day. A clinic-friendly translation is weightbanded “top-up” energy targets: a 6 kg child needs roughly 240–468 kcal/day from the supplement; an 8 kg child 320–624 kcal/day; and a 10 kg child 400–780 kcal/day. These are supplement calories, not total calories. Product intent matters. Ready-to-use therapeutic food (##RUTF##) is designed for treatment of severe wasting and/or nutritional oedema, not routine ##MAM## supplementation. Ready-to-use supplementary food (##RUSF##) and other lipid-based nutrient supplements are designed for moderate wasting; ##UNICEF## specifications describe ##RUSF## as eaten directly from the sachet without cooking or dilution, typically as one 100 g sachet per day providing about 535 kcal, and not as a breast-milk substitute. When used, prescribe a defined course (often about 8–12 weeks in programme models) and reassess early for adherence and displacement. Safe-use counselling prevents common operational failures. Give the supplement after the child has eaten home food, or split into two small post-meal portions, to reduce displacement. Ask specifically about sharing; encourage a “child portion first” rule. Store packets sealed away from heat and pests; once opened, avoid prolonged storage. Do not mix with water, which increases contamination risk. Screen for prior immediate food allergy reactions and supervise young children while eating thick pastes to reduce choking risk.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- UNICEF Supply Division. Ready-to-use supplementary food (RUSF) specification — https://www.unicef.org/supply/media/16636/file/S0000248-RUSF-Specification.pdf

- Indian Academy of Pediatrics. Integrated management of severe acute malnutrition: IAP consensus statement (2013) — https://www.indianpediatrics.net/apr2013/399.pdf

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;SUPPLEMENTARY FEEDING REGIMENS AND SAFE PRODUCT USE;FOOD SAFETY COUNSELLING FOR THR AND HOT COOKED MEALS;

Supplementary feeding fails when food is unsafe or when caregivers interpret coincidental diarrhoea as “the ration caused illness.” Food safety counselling should therefore be explicit and practical, aligned with programme quality expectations. For hot cooked meals (##HCM##), advise families to feed as fresh as possible. Avoid keeping cooked food at room temperature for prolonged periods; if food is stored, reheat thoroughly before feeding. For take-home rations (##THR##), instruct caregivers to store packets in a dry, covered container, use clean and dry utensils, and prepare with safe water. Handwashing with soap before preparation and feeding should be part of the

prescription, because contaminated water and hands can turn nutritional support into repeated diarrhoea. Discard the ration if there is a foul smell, visible insects, or moisture clumping. When diarrhoea occurs, counsel that feeding should continue. Use standard low-osmolarity ##ORS## for rehydration in ##MAM## (unless severe wasting or oedema is present) and give zinc for acute diarrhoea according to ##WHO## guidance (20 mg daily for 10–14 days, or 10 mg daily if under 6 months). Framing rehydration and continued feeding as part of the nutrition plan reduces the cycle of “illness leading to reduced intake leading to further wasting.” At follow-up, audit preparation: ask the caregiver to describe or show how the ration is prepared and how much the child actually eats. If repeated diarrhoea is linked to unsafe preparation or spoiled packets, treat it as a system and hygiene problem and help the family escalate to the anganwadi supervisor for replacement. This approach protects both clinical outcomes and caregiver trust in supplementary feeding.

References:

- Government of India. 2017. Supplementary Nutrition Rules under ICDS — https://hpfoodcommission.nic.in/pdf/Supplementary_Nutrition%20_ICDS_%20Rules_2017.pdf

- Ministry of Women and Child Development. Mission Saksham Anganwadi and Poshan 2.0 scheme guideline — https://wcd.delhi.gov.in/sites/default/files/WCD/generic_multiple_files/final_saksham_anganwadi_and_mission.pdf

- National Health Mission. Integrated management of neonatal and childhood illness guidelines (2 months to 5 years) — https://www.nhm.gov.in/images/pdf/guidelines/nrhmguidelines/stg/imnci-2-months-to-5-years.pdf

- World Health Organization. Zinc supplementation in diarrhoea (eLENA) — https://www.who.int/tools/elena/interventions/zinc-diarrhoea

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;FOLLOW-UP ALGORITHMS AND ESCALATION PATHWAYS;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;FOLLOW-UP ALGORITHMS AND ESCALATION PATHWAYS;RISK-STRATIFIED FOLLOW-UP SCHEDULING AND RESPONSE ASSESSMENT;

Follow-up is the core of outpatient ##MAM## care because moderate wasting can deteriorate quickly during intercurrent illness, and because early non-response often reflects correctable feeding or program failures. Start with a structured day 0 plan: document baseline anthropometry (weight, length/height when feasible, oedema, and at least one of ##MUAC## or ##WHZ##/##WLZ##), note which metric defined eligibility, identify dietary and health drivers, and link the child to a reliable supplementary feeding pathway. Decide follow-up intensity by risk. Use ##WHO## risk criteria (for example ##MUAC## 115–119 mm, ##WAZ## < −3, age <24 months, relapse, comorbidity, or severe social adversity) to classify the child as high-risk versus standard-risk. In many outpatient Indian settings, a feasible approach is review at about 7 days for high-risk children and within 14 days for

standard-risk children, with earlier review if there is any acute illness. At follow-up, the key question is trajectory: is the child clinically well, eating better, and showing consistent anthropometric improvement with no red flags? If improving, reinforce the simplest plan that the family can sustain and gradually space visits to fortnightly or monthly in coordination with community monitoring. If not improving, do not default to “change the supplement”; first re-check measurements, intake, sharing, and untreated disease, and intensify follow-up while correcting the identified failure points. State models summarized in ##NITI## and ##UNICEF## compendia commonly use weekly or fortnightly visits during an active programme period (often up to about 8 weeks) and then monthly monitoring after discharge. Using similar time anchors in clinic notes makes your plan compatible with what frontline workers can deliver.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

- National Health Mission. Operational guidelines on Facility Based Management of Children with SAM (FBMC) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/operational_guidelines_on_fbmc_with_sam.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;FOLLOW-UP ALGORITHMS AND ESCALATION PATHWAYS;DAY ZERO WEEKLY AND MONTHLY DECISION NODES FOR OUTPATIENT FOLLOW-UP;

Write follow-up as dated decision nodes so deterioration is not missed. Day 0 is the baseline: confirm ##MUAC## and/or ##WHZ##/##WLZ## with oedema check, record appetite and any danger signs, document the likely dietary and illness drivers, and link the child the same day to supplementary feeding entitlements. Classify as high-risk when any ##WHO## prioritization factor is present (##MUAC## 115–119 mm, ##WAZ## < −3, age under 24 months, relapse, prior ##SAM##, comorbidity or disability, or severe social adversity) and schedule review in 7 days; otherwise review in 14 days.

- Day 7 or day 14 is the first trajectory node. Repeat anthropometry with technique verification and complete an adherence audit (meals achieved, supplement received and actually consumed, dilution, and sharing). If there is weight loss, falling ##MUAC##, new oedema, poor appetite, or any clinical deterioration, treat this as escalation today: repeat triage and refer or admit if complicated wasting is suspected. If there is no deterioration but there is no improvement in appetite or trajectory, correct operational failures first and start symptom-guided screening for illness (for example persistent diarrhoea, ##UTI##, ##TB##, anemia) rather than changing foods immediately.

- Week 4 is the response node. If clinically well with improving appetite and upward anthropometry, continue the same plan and space follow-up while maintaining programme linkage. If non-responding at week 4 despite verified adherence, start staged investigations and consider referral when comorbidity or safeguarding concerns exist.

- Week 8 is the failure threshold. Persistent non-response by week 8 should be treated as failure of the outpatient pathway and referred for facility-based assessment (##NRC##/##FBMC## or paediatric ward) for comprehensive feeding and medical evaluation.

References:

- Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- Operational guidelines on facility-based management of children with severe acute malnutrition (FBMC) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/operational_guidelines_on_fbmc_with_sam.pdf

- Community-based management of acute malnutrition in India: a report (NITI Aayog and UNICEF) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Aayog.pdf

- IMNCI: standard treatment guidelines for 2 months to 5 years — https://www.nhm.gov.in/images/pdf/guidelines/nrhm-guidelines/stg/imnci-2-monthsto5years.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;FOLLOW-UP ALGORITHMS AND ESCALATION PATHWAYS;VISIT CHECKLIST AND ESCALATION CRITERIA DURING FOLLOW-UP;

Every outpatient ##MAM## follow-up visit should generate a minimum dataset that makes deterioration and operational failure visible. Document: anthropometry (weight, length/height when feasible, oedema check, ##MUAC## and/or ##WHZ##/##WLZ## with repeat if borderline); appetite and feeding function ("is the child eating more than last visit," who feeds, and how long meals take); morbidity screen (diarrhoea, cough or breathing difficulty, fever, and any danger signs); and supplement use audit when any ration or product is provided (amount received, amount consumed by the child, sharing, storage, and preparation errors). End the note with the single agreed "one change," the exact foods and schedule until next visit, and the next follow-up date. Escalate promptly when risk increases. Immediate referral or admission is required if nutritional oedema develops, any emergency or general danger sign appears, or there is clinical deterioration. During active follow-up, treat weight loss or a falling ##MUAC## as a red flag: reassess for untreated illness, inadequate intake, or unsafe home care, and refer for higher-level assessment when complications are suspected. State protocols summarized in Indian compendia explicitly use complications or weight loss to trigger referral to facility-based centres such as Nutrition Rehabilitation Centres (##NRC##). When specially formulated foods are unavailable, treat ##WHO## high-risk criteria as a marker for

“more frequent contact plus lower threshold to investigate and refer,” rather than as a reason to do less.

References:

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/202312/CMAM-report-NITI-Ayog.pdf

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- National Health Mission. Operational guidelines on Facility Based Management of Children with SAM (FBMC) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/operational_guidelines_on_fbmc_with_sam.pdf

- National Health Mission. Strengthening facility based paediatric care operational guidelines (ETAT triage) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/Strenghtening_Facility_Based_Paediatric_Care-Operational_Guidelines.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;FOLLOW-UP ALGORITHMS AND ESCALATION PATHWAYS;REFERRAL CRITERIA AND HANDOVER DOCUMENTATION FOR ESCALATION TO FACILITY CARE;

Escalation from outpatient moderate wasting must be explicit so children who deteriorate are not kept in “counselling only” follow-up. Move off the outpatient pathway immediately if bilateral pitting oedema appears, if severe wasting thresholds are reached (##MUAC## under 115 mm and/or ##WHZ##/##WLZ## under −3), or if appetite becomes poor on clinical assessment. Also escalate for any complication needing facility care: persistent vomiting, severe dehydration or shock, hypothermia, high fever with toxicity, respiratory distress, severe palmar pallor, extensive skin or eye lesions, or any situation where home care is unsafe. In India, refer to the nearest ##NRC## or paediatric ward implementing ##FBMC## operational guidance for stabilization and nutrition rehabilitation. After stabilization, ensure continuity of supplementary feeding and monitoring through local community pathways (##ICDS## entitlements via the ##AWC##, and ##CMAM## workflows where implemented). Do not wait for transport to start simple pre-referral actions: prevent hypoglycaemia with urgent feeding if safe, keep the child warm, treat dehydration cautiously and refer if severe, and give first-dose antibiotics only when a clear severe infection syndrome is present and local protocols support it without delaying referral. A handover note prevents information loss. Include dated anthropometry (weight, length/height if available, ##MUAC##, oedema), appetite status, feeding dysfunction observations (cough or choking with feeds, prolonged meals), the reason for referral, treatments already given, supplement exposure (##THR##/##HCM## or product received and actually consumed), and a brief social risk screen (food insecurity, caregiver capacity, safeguarding concerns).

References:

- Strengthening facility-based paediatric care: operational guidelines — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/Strenghtening_Facility_Based_Paediatric_Care-Operational_Guidelines.pdf

- Operational guidelines on facility-based management of children with severe acute malnutrition (FBMC) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/operational_guidelines_on_fbmc_with_sam.pdf

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- Community-based management of acute malnutrition in India: a report (NITI Aayog and UNICEF) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Aayog.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;FOLLOW-UP ALGORITHMS AND ESCALATION PATHWAYS;CASE VIGNETTES ILLUSTRATING FOLLOW-UP AND REFERRAL DECISIONS;

Vignette 1: discordant ##MUAC## and ##WHZ##. A 22-month child has ##MUAC## 121 mm but ##WHZ## just above −2 with recurrent diarrhoea. After repeating measurements, manage using the more severe risk marker and ##WHO## high-risk criteria (age under 24 months and low ##MUAC## range), schedule early review, and prioritize diarrhoea evaluation plus supplementary feeding rather than discharging because one index is near-normal. Vignette 2: non-response driven by sharing and dilution. A 2-year child's weight plateaus at the second visit and the caregiver reports the ##THR## "finishes in 3 days." An adherence audit reveals household sharing and preparation as watery gruel. The clinical intervention is demonstration of energy-dense preparation, negotiation of an "index child share" plan, and supervised follow-up through the ##AWC##/##VHSND## system, not switching products. Vignette 3: moderate wasting with ##TB## suspicion. A 3-year child with ##MUAC## 117 mm has cough and fever for 3 weeks and a known household contact. Classify as high-risk, start the ##NTEP## diagnostic pathway and refer early, while continuing nutrition rehabilitation and close follow-up. Vignette 4: relapse within 2 months. A child discharged with ##MUAC## 125 mm returns 6 weeks later with ##MUAC## 123 mm after two diarrhoeal episodes. Treat relapse as a marker of unresolved drivers: restart an intensified follow-up schedule, ensure uninterrupted supplementary feeding access, and re-check immunization, deworming, and ##WASH## counselling.

References:

- Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- Community-based management of acute malnutrition in India: a report (NITI Aayog and UNICEF) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Aayog.pdf

- Paediatric TB management guideline (NTEP) — https://tbcindiawp.azurewebsites.net/wpcontent/uploads/2023/05/9534339438Paediatric_TB_Mangement_Guideline_22082022_V1.pdf

- Supplementary Nutrition (ICDS) Rules, 2017 — https://hpfoodcommission.nic.in/pdf/Supplementary_Nutrition%20_ICDS_%20Rules_2017.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;DISCHARGE CRITERIA AND RELAPSE PREVENTION AFTER RECOVERY;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;DISCHARGE CRITERIA AND RELAPSE PREVENTION AFTER RECOVERY;SUSTAINED RECOVERY CRITERIA AND DISCHARGE PLANNING;

Exit criteria in wasting care must prioritize stability. The 2023 ##WHO## wasting guideline discourages using percentage weight gain or absolute weight gain as outpatient exit criteria and instead requires sustained anthropometric recovery. A conservative, clinic-safe recovery definition for children treated for ##MAM## is: the child is clinically well and eating adequately, has no nutritional oedema, and shows sustained anthropometric recovery on consecutive measurements. Use dual criteria when feasible to reduce the risk of “exiting by one metric while still eligible by another.” Define recovery as ##MUAC## ≥125 mm and ##WHZ##/##WLZ## ≥ −2 maintained for at least two consecutive visits, with no bilateral pitting oedema on at least two consecutive visits. If only one metric can be measured reliably, base discharge on that metric but document the limitation and maintain a lower threshold for continued monitoring in high-risk children. Indian operational workflows may prioritize one metric for programme entry (often ##WHZ## plus oedema in community screening). To protect continuity across migration and referrals, record both ##MUAC## and ##WHZ##/##WLZ## at discharge when possible and state explicitly which measure was used for programme entry versus clinical monitoring. Discharge from clinic- led “active wasting follow-up” should always include a linkage plan for continued community growth monitoring rather than an open-ended “review if needed.” Before discharge, confirm that the feeding plan is feasible, the caregiver understands how to use any ration, and the next monitoring touchpoint is scheduled (clinic review or community monitoring through ##AWC## or ##VHSND##).

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

- National Health Mission. Operational guidelines on Facility Based Management of Children with SAM (FBMC) —

https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/operational_guidelines_on_fbmc_with_sam.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;DISCHARGE CRITERIA AND RELAPSE PREVENTION AFTER RECOVERY;DEFINING TREATMENT FAILURE RELAPSE AND POST-RECOVERY MONITORING;

Define failure and relapse explicitly so follow-up becomes decision-driven. Early failure (within 1–2 weeks) includes any clinical deterioration (new emergency or danger sign) or anthropometric worsening (weight loss or falling ##MUAC##). This should trigger urgent illness management, repeat triage, and referral or admission if complications are present. Define non-response at structured follow-up (after two visits, typically about 2–4 weeks) as no meaningful improvement in appetite or feeding behaviour and no upward anthropometric trajectory despite verified adherence to the feeding and supplementation plan. This should trigger an operational audit (measurement technique, sharing, displacement, preparation errors) and a targeted medical evaluation rather than simply changing products. Some Indian state protocols use an active follow-up window of about 8 weeks for ##MAM##; persistent non-response by this time is a practical threshold to refer to a higher facility for comprehensive assessment. Relapse is clinically meaningful because it signals unresolved household food insecurity, recurrent infection burden, or unaddressed comorbidity. Indian facility-based malnutrition guidance defines relapse operationally as re-eligibility within about 2 months after cure. A clinic-ready relapse definition is: within 2 months of discharge, the child again meets moderate wasting criteria (##MUAC## 115–124 mm and/or ##WHZ##/##WLZ## −3 to −2) or shows clinical deterioration suggesting risk of progression. Use a graduated post-recovery monitoring schedule aligned to service touchpoints: one review in the first month after recovery (preferably through ##VHSND## or ##AWC## growth monitoring), monthly monitoring for months 2–3 in high-risk children, and at least one additional review between months 4–6 for children under 24 months or with major social vulnerability.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

- National Health Mission. Operational guidelines on Facility Based Management of Children with SAM (FBMC) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/operational_guidelines_on_fbmc_with_sam.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;DISCHARGE CRITERIA AND RELAPSE PREVENTION AFTER RECOVERY;RESPONSIVE CAREGIVING AND EARLY LEARNING COUNSELLING DURING RECOVERY MONITORING;

Nutrition recovery depends on how children are fed, not only what is offered. Build counselling around responsive caregiving: the caregiver decides what foods are offered and when, while the child decides how much to eat. Encourage active, patient feeding (sitting with the child, eye contact, gentle prompting) and discourage coercion, distraction by screens, or prolonged battles that reduce intake and increase caregiver stress.

- A brief script that fits a busy clinic is: “Sit the child upright and close. Offer thick food slowly. Watch for cues; if the child refuses, pause and try again, but do not force. Praise any attempt to eat.” For children with low appetite after illness, recommend smaller, more frequent meals and reassurance that appetite improves as infections resolve.

- Psychosocial stimulation supports growth and reduces relapse risk. Advise caregivers to add “play and talk” routines to meals and daily care: naming objects, singing, and simple games. For toddlers, encourage self-feeding with finger foods to improve autonomy and intake. For preschoolers, consistent routines and limiting excessive screen time can improve appetite regulation and sleep.

- When there are clear caregiving constraints (maternal depression, severe family stress, substance use, neglect), treat this as a clinical risk factor for non-response and relapse. Link families to community support through ##ICDS## home visits, ##AWC## platforms, and available maternal mental health or social services. Document one achievable caregiving action to review at the next follow-up, just as you document a diet change.

References:

- Nurturing Care Framework for Early Childhood Development (WHO, UNICEF, World Bank, 2018) — https://apps.who.int/iris/bitstream/handle/10665/272603/9789241514064eng.pdf?ua=1

- Responsive feeding is embedded in a theoretical framework of responsive parenting — https://pmc.ncbi.nlm.nih.gov/articles/PMC3040905/

- Enhancing optimal infant and young child feeding practices: operational guidelines — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/EnhancingoptimalIYCFpractices.pdf

- Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;NON-RESPONSE EVALUATION AND TARGETED INVESTIGATION;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;NON-RESPONSE EVALUATION AND TARGETED INVESTIGATION;DISTINGUISHING NON-RESPONSE FROM ADHERENCE FAILURE;

When a child with ##MAM## is not improving, start by confirming that this is true non -response rather than measurement or adherence failure. At the second follow -up (commonly 2–4 weeks), repeat anthropometry with correct technique, because discordant measures and small errors are common. Audit actual intake in detail: who feeds, how long meals take, what the child actually eats, and whether any supplement is shared or displaces family foods. Re-screen for acute illness and danger signs; an outpatient child can deteriorate quickly. If adherence is poor, treat the operational failure first and intensify follow-up instead of ordering broad panels. Common correctable failures include: ration mixed into watery tea or thin gruels, sharing among siblings, unsafe storage, and the caregiver's misconception that solids "cause diarrhoea," leading to restriction. Move to targeted evaluation when there are red flags or when non-response persists despite verified adherence. Trigger immediate evaluation when there is chronic or recurrent diarrhoea, persistent vomiting, abdominal distension, oedema, chronic fatigue, recurrent fever, chronic cough, or known contact with ##TB##. Also consider congenital heart disease when there is tachypnoea with feeds, sweating, recurrent chest infections, or a murmur, and consider feeding dysfunction or disability when mealtimes are prolonged and intake is consistently low. These conditions are explicitly recognized as comorbidities that increase risk and justify more intensive support in ##WHO## guidance.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- National Tuberculosis Elimination Programme. Paediatric TB management guideline (2022) — https://tbcindia-wp.azurewebsites.net/wpcontent/uploads/2023/05/9534339438Paediatric_TB_Mangement_Guideline_22082022_V1.pdf

- Indian Council of Medical Research. Guideline for diagnosis and management of celiac disease in India — https://www.icmr.gov.in/icmrobject/custom_data/pdf/resourceguidelines/ICMR%20-%20Diagnosis%20and%20Managmemnt.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;NON-RESPONSE EVALUATION AND TARGETED INVESTIGATION;CLINICAL CLUSTERS SUGGESTING TUBERCULOSIS CELIAC OR CARDIAC DISEASE;

Treat non-response and early relapse as "symptoms" that require a directed search for an underlying driver, not as a reason to keep changing supplements. Use clinical clusters to trigger targeted evaluation early, especially when adherence is verified or when systemic symptoms are present.

- A ##TB## suspicion cluster includes cough for 2 weeks or more, fever for 2 weeks or more, reduced playfulness, weight loss or failure to thrive, known household contact, lymphadenopathy, or persistent pneumonia or pleural effusion. In a child with wasting plus this cluster, combine nutrition rehabilitation with immediate linkage to the ##NTEP## diagnostic pathway (chest imaging and microbiology with ##NAAT## where feasible), rather than waiting for “week 8 failure.”

- A celiac disease cluster includes chronic diarrhoea or constipation with poor weight gain, abdominal distension, anemia not responding to iron, recurrent mouth ulcers, or a compatible family history. If suspicion is high, start serology-based evaluation per Indian guidance and avoid empiric long-term dietary restriction without diagnosis.

- A cardiac or chronic lung disease cluster includes tachypnoea at rest, feeding fatigue, diaphoresis with feeds, recurrent lower respiratory infections, or poor linear growth with poor weight gain. These children should be risk-stratified as high-risk wasting, have careful cardiopulmonary examination, and be referred for appropriate imaging or echocardiography when indicated.

- A chronic infection or ##UTI## cluster includes recurrent fever, unexplained vomiting, and poor weight gain (especially in infants), even without urinary symptoms. Malabsorption or parasitic disease is suggested by persistent loose stools, steatorrhoea, perianal excoriation, or pica. Use these clusters to sequence investigations and referrals while maintaining close follow-up.

References:

- Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- Paediatric TB management guideline (NTEP) — https://tbcindiawp.azurewebsites.net/wpcontent/uploads/2023/05/9534339438Paediatric_TB_Mangement_Guideline_22082022_V1.pdf

- Diagnosis and management of celiac disease in children (ICMR resource guideline) — https://www.icmr.gov.in/icmrobject/custom_data/pdf/resourceguidelines/ICMR%20%20Diagnosis%20and%20Managmemnt.pdf

- Standard treatment guidelines: urinary tract infection in children (IAP) — https://iapindia.org/pdf/STG-06-Urinary-Tract-Infection-in-Children.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;NON-RESPONSE EVALUATION AND TARGETED INVESTIGATION;STAGED WORK-UP FOR COMMON MEDICAL CAUSES;

A staged investigation strategy avoids both under-diagnosis and low-yield testing. After verified adherence and persistent non-response, start with high-yield tests that change management. Stage A basic tests (often appropriate around 2–4 weeks of non-response): haemoglobin to grade anaemia and identify severe anaemia risk; stool examination when diarrhoea is persistent to look for parasites or blood; and urinalysis with culture when there is recurrent fever or poor growth without a clear source to detect occult ##UTI##. Choose additional tests based on clinical examination (for example, chest radiograph only when respiratory examination suggests lower respiratory disease or ##TB##). Stage B targeted tests (often appropriate around 4–8 weeks, or earlier if red flags are present): for suspected celiac disease, follow ##ICMR## guidance that IgA anti-tTG is the preferred serologic test in children above 2 years; if suspicion remains high with negative serology, exclude selective IgA deficiency and use IgG-based tests (IgG anti-tTG or IgG anti-##DGP##) when IgA deficiency is present. For suspected ##TB##, follow ##NTEP## paediatric algorithms, using chest radiography and bacteriological confirmation pathways, including ##NAAT##/##CBNAAT## where available. Consider ##HIV## testing when clinically indicated or per local policy, especially with recurrent infections or persistent poor response. Escalate disposition at any point if a new danger sign appears, if the child develops oedema, or if severe illness is suspected. Persistent non-response despite operational correction by about 8 weeks should prompt referral to a higher facility for comprehensive evaluation.

References:

- Indian Council of Medical Research. Guideline for diagnosis and management of celiac disease in India — https://www.icmr.gov.in/icmrobject/custom_data/pdf/resourceguidelines/ICMR%20-%20Diagnosis%20and%20Managmemnt.pdf

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- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;NON-RESPONSE EVALUATION AND TARGETED INVESTIGATION;INTERPRETATION THRESHOLDS AND REFERRAL TRIGGERS FOR COMMON INVESTIGATIONS;

Targeted tests only help if residents know what results change disposition. For anemia, treat severe palmar pallor as a referral trigger in ##IMNCI## logic. Where hemoglobin testing is available, use severity to guide urgency: severe anemia by ##WHO## classification (hemoglobin under 7 g/dL for ages 6–59 months) or any anemia with cardiorespiratory symptoms should prompt facility evaluation for cause and need for transfusion rather than outpatient iron alone. For occult ##UTI##, correct

sampling is more important than “ordering urine.” In non– toilettrained children, use catheterization or suprapubic aspiration for culture; avoid urine bags because contamination produces false positives. Interpret culture thresholds by method (suprapubic aspiration: any growth; catheter: greater than 10^4 ##CFU##/mL; clean-catch midstream: greater than 10^5 ##CFU##/mL) and process samples promptly. Treat confirmed ##UTI## and plan follow-up because recurrent infection perpetuates poor weight gain. For celiac disease, avoid shortcuts. Indian guidance anchors diagnosis on compatible serology plus histology, and emphasizes that mild intraepithelial lymphocytosis alone is not specific. If clinical suspicion is high, ensure appropriate serology (including evaluation for ##IgA## deficiency when needed) and refer for definitive work-up rather than starting a long-term gluten-free diet without confirmation. For ##TB##, a safety rule is: wasting plus cough or fever for 2 weeks or more, or known contact, requires parallel nutrition support and immediate linkage to the ##NTEP## diagnostic pathway; nutrition escalation without screening is unsafe.

References:

- Haemoglobin concentrations for the diagnosis of anaemia and assessment of severity (WHO) — https://www.who.int/publications/i/item/WHO-NMH-NHD-MNM-11.1

- IMNCI: standard treatment guidelines for 2 months to 5 years — https://www.nhm.gov.in/images/pdf/guidelines/nrhm-guidelines/stg/imnci-2-monthsto5years.pdf

- Standard treatment guidelines: urinary tract infection in children (IAP) — https://iapindia.org/pdf/STG-06-Urinary-Tract-Infection-in-Children.pdf

- Diagnosis and management of celiac disease in children (ICMR resource guideline) — https://www.icmr.gov.in/icmrobject/custom_data/pdf/resource-guidelines/ICMR%20-%20Diagnosis%20and%20Managmemnt.pdf

- Paediatric TB management guideline (NTEP) — https://tbcindiawp.azurewebsites.net/wpcontent/uploads/2023/05/9534339438Paediatric_TB_Mangement_Guideline_22082022_V1.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;PROGRAM LINKAGES AND CONTINUUM OF CARE IN INDIA;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;PROGRAM LINKAGES AND CONTINUUM OF CARE IN INDIA;CLINIC WORKFLOW FOR LINKING TO ANGANWADI AND COMMUNITY SERVICES;

Programme linkage is a clinical intervention: diagnosing ##MAM## without connecting the child to entitlements and community monitoring leads to preventable failure. Mission Poshan scheme guidance specifies that beneficiaries must be registered at the nearest anganwadi centre and that supplementation is delivered through entitlements such as ##THR## and ##HCM##, typically for a minimum of about 300 days per year. It also describes a migration facility in the Poshan Tracker to maintain access when families temporarily move. A clinic-ready linkage ##SOP## is: (1) the pediatrician or resident confirms uncomplicated ##MAM## using triage criteria, documents

anthropometry, and writes a clear outpatient plan; (2) the clinic issues a written linkage note to the caregiver naming the anganwadi and stating what the child is entitled to, plus the clinic follow-up date; (3) the anganwadi worker (##AWW##) registers or updates the child and arranges ration or meals, with supervisor support if denial occurs; and (4) health staff (##ANM##/##ASHA##) support morbidity screening and referral, often through monthly ##VHSND## touchpoints where growth monitoring occurs. Align clinic review with community cadence when feasible to reduce travel burden. Write referral notes so they can be acted on. Include child identifiers and contact, date-stamped anthropometry (weight, length/height, ##MUAC##, oedema status), a one-line clinical status ("no danger signs today" or specify concerns), the outpatient plan and next clinic date, and a requested action ("ensure registration, provide age-appropriate ##THR##/##HCM##, schedule growth monitoring, and report if ration not received"). Clear notes reduce exclusion due to administrative ambiguity.

References:

- Ministry of Women and Child Development. Mission Saksham Anganwadi and Poshan 2.0 scheme guideline — https://wcd.delhi.gov.in/sites/default/files/WCD/generic_multiple_files/final_saksham_anganwadi_and_mission.pdf

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

- Government of India. 2017. Supplementary Nutrition Rules under ICDS — https://hpfoodcommission.nic.in/pdf/Supplementary_Nutrition%20_ICDS_%20Rules_2017.pdf

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;PROGRAM LINKAGES AND CONTINUUM OF CARE IN INDIA;TROUBLESHOOTING ENTITLEMENTS MIGRATION AND QUALITY FAILURES;

Linkage fails at predictable points; troubleshooting should be part of routine counselling. If benefits are denied due to migration, document the temporary address and request migration-based registration through the Poshan Tracker facility described in Mission Poshan guidance. If ration is not received, is inconsistent, or appears poor quality, document dates and packet counts, advise the caregiver to report the issue, and escalate to the anganwadi supervisor or ##CDPO##. Programme rules and guidance emphasize quality assurance and compliance with food safety requirements, so quality complaints are not "optional." When a child cannot attend daily ##HCM## (distance, caregiver work), shift emphasis to predictable ##THR## access where applicable and intensify the household feeding prescription so the child still receives adequate energy density. If there is non-response, check operational causes first: ration sharing, incorrect preparation (dilution), or diversion of the child's portion. Treat these as fixable system problems before escalating medical

investigations. Clinically and ethically, supplementary nutrition is part of standard care for eligible children rather than a discretionary welfare add-on. Documentation protects access: accurate anthropometry, clear classification as ##MAM##, and written linkage notes reduce exclusion. Keep a clinic contact list of local anganwadi supervisors, the nearest Nutrition Rehabilitation Centre (##NRC##), and the local ##VHSND## day for your catchment area so families can be directed immediately to the correct platform.

References:

- Government of India. 2017. Supplementary Nutrition Rules under ICDS — https://hpfoodcommission.nic.in/pdf/Supplementary_Nutrition%20_ICDS_%20Rules_2017.pdf

- Ministry of Women and Child Development. Mission Saksham Anganwadi and Poshan 2.0 scheme guideline — https://wcd.delhi.gov.in/sites/default/files/WCD/generic_multiple_files/final_saksham_anganwadi_and_mission.pdf

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

- National Health Mission. Operational guidelines on Facility Based Management of Children with SAM (FBMC) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/operational_guidelines_on_fbmc_with_sam.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;FEEDING RESTORATION PATHWAY FOR INFANTS UNDER SIX MONTHS;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;FEEDING RESTORATION PATHWAY FOR INFANTS UNDER SIX MONTHS;TRIAGE CRITERIA AND BREASTFEEDING RESTORATION UNDER SIX MONTHS;

Acute malnutrition in infants under 6 months should not be managed by simply extending olderchild supplementary feeding logic. The 2023 ##WHO## wasting guideline treats infants under 6 months “at risk of poor growth and development” as a high-risk group with explicit admission and outpatient criteria, and emphasizes assessment of the mother–infant dyad. Refer and admit infants under 6 months when any of the following are present: any ##IMCI## danger sign; any acute medical problem under a severe classification; nutritional oedema; or recent weight loss. Even when these are absent, inpatient or structured outpatient care may be needed when anthropometry indicates significant risk (for example ##WAZ## < −2, ##WLZ## < −2, or ##MUAC## <110 mm for ages 6 weeks to under 6 months) or when there is failure to gain weight on two consecutive measurements. Outpatient management is appropriate only when no danger signs are present and admission criteria are not met, and the caregiver can return reliably. Feeding restoration is the core intervention. Observe a breastfeed in clinic and document positioning, attachment, and transfer cues (deep slow

sucks, swallowing, satiety cues, and urine output). Intensify lactation support and maternal support before introducing supplementary milk, because ##WHO## states that decisions about supplementary milk in addition to breastfeeding must be based on comprehensive assessment of infant needs and maternal physical and mental health. For infants discharged from inpatient care, transfer to outpatient follow-up only after at least 48 hours without danger signs, resolution of acute problems, resolving oedema, good appetite, and documented weight gain for 2–3 days on exclusive breastfeeding or appropriate replacement feeding, with clear linkage to follow-up support. Reduce visit frequency only after sustained weight gain for at least two consecutive weekly visits.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- National Health Mission. Integrated management of neonatal and childhood illness guidelines (2 months to 5 years) — https://www.nhm.gov.in/images/pdf/guidelines/nrhmguidelines/stg/imnci-2-months-to-5years.pdfNationalHealthMission.Guidelinesforenhancingoptimalinfantandyoungchildfeedingpractices—https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/Enhancingoptimal-IYCF-practices.pdf

- National Health Mission. MAA programme trainers guide for breastfeeding and lactation support — https://nhm.gov.in/New_Updates_2018/NHM_Components/RMNCHA/CH/Schemes/Maa/TRAINERS%27_GUIDE_FINAL_Web.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;FEEDING RESTORATION PATHWAY FOR INFANTS UNDER SIX MONTHS;DYAD ASSESSMENT SUPPLEMENTATION OPTIONS AND FOLLOW-UP INTENSITY UNDER SIX MONTHS;

For infants under 6 months, the “patient” is the mother–infant dyad, and supplementation decisions must protect breastfeeding while preventing further weight loss. At baseline, document a dyad assessment: frequency of feeds (including night feeds), effective attachment and transfer on observed feed, maternal breast problems, maternal nutrition and illness, and barriers such as return to work or lack of support. If breastfeeding transfer is inadequate but the infant is clinically stable, prioritize supplementation that preserves breastfeeding. First choice is expressed breast milk offered by cup or spoon after breastfeeding, while intensifying relactation support and correcting positioning. If expressed milk is not available or the infant remains hungry, use age-appropriate infant formula prepared at correct concentration; counsel explicitly against dilution and against bottle contamination. Avoid introducing animal milk or thin cereal gruels as “bridges” because they increase infection risk and reduce nutrient density. Follow-up intensity should be higher than in 6–59 months. Recheck weight, hydration, urine output, and feeding effectiveness within 48–72 hours after the initial plan, then at 7 days, and then weekly until consistent weight gain and improved breastfeeding are documented. Escalate to facility-based care when there is any danger sign, poor feeding, dehydration, hypothermia, oedema, or lack of weight gain despite verified support, and

ensure linkage to skilled lactation support platforms (##MAA## counselling, ##FBMC## units, or paediatric services depending on local availability).

References:

- Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- Mothers' Absolute Affection (MAA): operational guidelines (MoHFW) — https://nhm.gov.in/MAA/Operational_Guidelines.pdf

- Enhancing optimal infant and young child feeding practices: operational guidelines — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/EnhancingoptimalIYCFpractices.pdf

- Infant and young child feeding counselling: an integrated course (WHO) — https://iris.who.int/bitstream/handle/10665/349471/9789240033874-eng.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;DIAGNOSIS AND MANAGEMENT OF THINNESS IN AGES 5 TO 19 YEARS;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;DIAGNOSIS AND MANAGEMENT OF THINNESS IN AGES 5 TO 19 YEARS;BMI-FOR-AGE CLASSIFICATION DIFFERENTIAL DIAGNOSIS AND PROGRAM LINKAGES;

Children older than 5 years are not classified with ##MUAC## and ##WHZ## the way 6–59 month children are. Use the ##WHO## 5–19 year growth reference: thinness is body mass index-for-age (##BAZ##) below −2, and severe thinness below −3. Always interpret ##BAZ## together with height-for-age because a short child may appear "less thin" by ##BMI## even with significant undernutrition. Clinical triage is still first. Escalate urgently if there are danger signs, rapid weight loss, dehydration, syncope, or suspected severe medical illness. In stable children, assess dietary intake (meal skipping, low protein foods, high consumption of tea and packaged snacks), recent illness, physical activity, menstruation history in adolescents, and psychosocial drivers including food insecurity, neglect, bullying, or disordered eating. Management mirrors the under-5 principles but uses age-appropriate platforms. Provide a quantified plan (three meals plus two nutritious snacks, daily protein source, and fat add-ons) and treat intercurrent illness. Screen early for common medical causes when thinness is persistent or severe: chronic diarrhoea or abdominal pain (celiac disease), chronic cough or fever (##TB##), recurrent infections, and urinary symptoms. Link families to government programmes that can improve intake and monitoring: school meals under ##PM####POSHAN##, adolescent platforms such as ##RKSK##, and anaemia prevention under ##WIFS##. Plan follow-up to document trajectory (weight and ##BMI## trend) rather than a single measurement.

References:

- BMI-for-age (5-19 years): interpretation of cut-offs (WHO growth reference) — https://www.who.int/tools/growth-reference-data-for-5to19-years/indicators/bmi-for-age

- RKSK Operational Framework (MoHFW, 2014) — https://nhm.gov.in/images/pdf/programmes/RKSK/RKSK_Operational_Framework.pdf

- Operational Framework: WIFS for Adolescents (MoHFW) — https://nhm.gov.in/images/pdf/programmes/wifs/operationalframeworkwifs/operational_framework_wifs.pdf

- Guidelines on PM POSHAN Scheme (Ministry of Education) — https://pmposhan.education.gov.in/Files/Guidelines/2023/Guidelines%20on%20PM%20POSHAN%20SCHEME.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;MANAGEMENT MODIFICATIONS IN COMORBIDITY AND SOCIAL ADVERSITY;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;MANAGEMENT MODIFICATIONS IN COMORBIDITY AND SOCIAL ADVERSITY;FOLLOW-UP AND REFERRAL MODIFICATIONS IN COMORBIDITY;

Children with ##MAM## and comorbidity have higher risk of deterioration and poorer recovery, so the care plan must be modified rather than simply intensified. The 2023 ##WHO## wasting guideline lists comorbidity (including ##HIV##, ##TB##, disability) and severe adverse social circumstances as factors that justify prioritizing specially formulated foods with counselling and tighter follow-up. In Indian practice, this translates into four concrete modifications. First, maintain a lower threshold for admission or referral when acute illness occurs, because physiology can deteriorate quickly even when anthropometry remains “moderate.” Second, schedule more frequent follow-up and extend monitoring beyond anthropometric recovery, particularly for children under 24 months, those with disability-related feeding dysfunction, and those with repeated infections. Third, start targeted evaluation earlier when there is non-response, because the pre-test probability of underlying disease is higher (for example chronic infection, malabsorption, cardiac disease, or neglect). Fourth, strengthen linkage beyond nutrition: connect to disease-specific programmes (##NTEP## for ##TB##, ##HIV## care services), rehabilitation and developmental services, and social support mechanisms for food security when available. Document these risk modifiers explicitly in the record. When supplements are used, counsel carefully to prevent displacement and to ensure safe feeding in children with swallowing difficulty or poor oral motor control. For children in severe social adversity, include a “caregiver reliability” assessment in disposition decisions, because inability to provide safe home care is itself a trigger for admission in facility-based guidance.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- National Tuberculosis Elimination Programme. Paediatric TB management guideline (2022) — https://tbcindia-wp.azurewebsites.net/wp-content/uploads/2023/05/9534339438Paediatric_TB_Mangement_Guideline_22082022_V1.pdf

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- National Health Mission. Operational guidelines on Facility Based Management of Children with SAM (FBMC) — https://nhm.gov.in/images/pdf/programmes/childhealth/guidelines/operational_guidelines_on_fbmc_with_sam.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;MANAGEMENT MODIFICATIONS IN COMORBIDITY AND SOCIAL ADVERSITY;FEEDING SAFETY AND TEXTURE MODIFICATION IN NEURODISABILITY OR ORAL-MOTOR DYSFUNCTION;

Disability and oral-motor dysfunction can make a standard high-calorie diet plan unsafe. Suspect feeding or swallowing difficulty when meals routinely exceed 30 minutes, the child coughs or chokes with liquids, has a “wet” voice after feeds, drools excessively, has recurrent chest infections, or has poor head and trunk control. In these children, weight faltering may reflect aspiration risk and feeding fatigue rather than inadequate food availability alone.

- A practical outpatient safety screen is to observe a real meal. Watch posture (upright, head midline), breathing effort during feeding, coordination of suck–swallow–breathe, and signs of distress (coughing, gagging, colour change). If unsafe signs appear, do not intensify thin liquids or force feeding. Modify feeding first: smaller, more frequent feeds; thicker textures that are easier to control; slow pacing with pauses; and supportive positioning. Counsel caregivers that the goal is safe swallowing plus energy density, not volume.

- Nutrition rehabilitation can still be pursued by concentrating calories in safe textures (thick khichdi with ghee, curd-based preparations, nut pastes if age-appropriate and allergy risk considered). When aspiration is suspected, or when growth does not improve despite safe modifications, escalate to specialist evaluation. In India, link early to ##RBSK## services and the ##DEIC## for multidisciplinary assessment (paediatrics, nutrition, physiotherapy/occupational therapy, and speech or feeding therapy where available). Document the feeding plan and follow-up at shorter intervals to reassess safety and caregiver technique.

References:

- Cerebral palsy in under 25s: assessment and management (NICE NG62) — https://www.nice.org.uk/guidance/ng62

- Operational Guidelines for DEIC under RBSK (NHM) — https://nhm.gov.in/images/pdf/programmes/RBSK/Operational_Guidelines/Operational-Guidelines-DEIC-RBSK.pdf

- Pediatric feeding and swallowing (ASHA Practice Portal) — https://www.asha.org/practiceportal/clinical-topics/pediatric-feeding-and-swallowing/

- Infant and young child feeding counselling: an integrated course (WHO) — https://iris.who.int/bitstream/handle/10665/349471/9789240033874-eng.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;PRACTICE VARIATIONS AND EMERGING EVIDENCE IN INDIA;

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;PRACTICE VARIATIONS AND EMERGING EVIDENCE IN INDIA;SCREENING METRIC VARIATION AND DOCUMENTATION STRATEGY;

Screening tool choice is a practical controversy in India because health and women-and-child systems may use different entry criteria. Medical definitions and the ##WHO## guideline include both ##MUAC## and ##WHZ##/##WLZ## for identifying moderate wasting. In contrast, Indian programme compendia note that many community workflows primarily use ##WHZ## plus oedema and that the Ministry of Women and Child Development (##MoWCD##) has not endorsed routine use of ##MUAC## for community screening in non-emergency settings, reserving it for emergencies such as pandemics or natural disasters. This can result in a child meeting clinical ##MAM## criteria by ##MUAC## but not being recognized by the local entitlement pathway. The clinical strategy is documentation plus risk-based care. When feasible, record both ##MUAC## and ##WHZ##/##WLZ## at diagnosis and follow-up. If the measures disagree, use the more severe measure for clinical risk stratification and follow-up intensity, while using the locally required measure for programme entry. Provide the caregiver with a written note that includes both measures and states that the child has acute wasting requiring supplementary nutrition support and monitoring. This reduces the risk that the child is dismissed due to a single metric. Teach residents that the metric is a tool, not the target. The treatment goal is sustained recovery and prevention of relapse, so decisions should be driven by clinical stability, appetite, illness burden, and trajectory over time rather than by a single measurement at one visit.

References:

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- PLOS ONE. MUAC and WHZ as predictors of mortality and programme outcomes in children under 5 years — https://journals.plos.org/plosone/article?id=10.1371%2Fjournal.pone.0219745

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;PRACTICE VARIATIONS AND EMERGING EVIDENCE IN INDIA;ANTIBIOTIC USE DECISIONS IN MODERATE WASTING;

Antibiotic decisions in ##MAM## should follow clinical infection syndromes rather than the nutrition label. Some community management programme models in India describe “antibiotics and deworming” as part of a package for acute malnutrition, which can create a perception that all children with moderate wasting should receive routine antibiotics. However, the 2023 ##WHO## wasting guideline emphasizes comprehensive assessment and treatment of medical problems contributing to moderate wasting, and does not recommend routine antibiotics for all children with ##MAM## in the same way that standardized antibiotic protocols are used in severe wasting. Apply antibiotic stewardship principles. Prescribe antibiotics only when there is a clinical diagnosis that warrants treatment (for example pneumonia, dysentery, otitis media, skin and soft tissue infection, or suspected sepsis), and use national standard treatment guidelines for drug choice, dose, and duration. Avoid “nutrition-only antibiotics” given to improve weight gain without evidence of bacterial infection, because this exposes children to adverse effects and increases antimicrobial resistance without clear benefit. Use the ##MAM## follow-up structure to detect infection early instead of pre-emptive prescribing. At each visit, screen for cough or fast breathing, diarrhoea with blood, fever, urinary symptoms, and ear discharge, and reassess triage status. When infections are recurrent or severe, treat the acute episode appropriately and then evaluate for underlying drivers such as ##TB##, ##HIV##, crowded housing, or poor ##WASH##, rather than repeating empirical antibiotic courses.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- National Health Mission. Integrated management of neonatal and childhood illness guidelines (2 months to 5 years) — https://www.nhm.gov.in/images/pdf/guidelines/nrhmguidelines/stg/imnci-2-months-to-5-years.pdf

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

(MAM) MODERATE ACUTE MALNUTRITION AND SUPPLEMENTARY FEEDING;PRACTICE VARIATIONS AND EMERGING EVIDENCE IN INDIA;POST-EXIT INTERVENTIONS TO PREVENT RELAPSE;

Preventing relapse requires planning beyond the point when anthropometry crosses a threshold. The 2023 ##WHO## wasting guideline recommends post-exit interventions for children who recover from

wasting, including continued nutrition counselling, improved ##WASH##, responsive caregiving, and psychosocial stimulation. This reflects growing evidence that relapse and developmental outcomes are influenced by household infection exposure and caregiving quality, not calories alone. In clinic, define discharge as a bundle: sustained anthropometric recovery plus a feasible feeding plan plus programme linkage plus a scheduled reassessment. After recovery, schedule monitoring around predictable vulnerability periods (for example the next diarrhoea season) and use community touchpoints such as ##VHSND## and anganwadi growth monitoring to maintain surveillance with low travel burden. Recurrent relapse should be treated as a signal to reassess food insecurity, ration access, hygiene, and comorbidity, rather than repeating short-term feeding cycles alone. Counsel caregivers that recovery is a phase, not an event. Encourage continued energy-dense family foods for several months, maintain routine micronutrient and deworming schedules, and promote simple stimulation activities (talking, play, responsive interaction) alongside feeding, especially for children under 24 months. These low-cost post-exit supports are often the difference between sustained recovery and repeated deterioration. When services allow, add a brief screen for caregiver stress and maternal depression, because impaired caregiving can undermine feeding, timely care-seeking, and adherence to follow-up.

References:

- World Health Organization. 2023. Guideline on the prevention and management of wasting and nutritional oedema in infants and children under 5 years — https://www.ncbi.nlm.nih.gov/books/NBK601642/

- NITI Aayog and UNICEF. 2023. Transforming child nutrition in India through Community-Based Management of Acute Malnutrition (CMAM) — https://www.niti.gov.in/sites/default/files/2023-12/CMAM-report-NITI-Ayog.pdf

- Ministry of Women and Child Development. Mission Saksham Anganwadi and Poshan 2.0 scheme guideline — https://wcd.delhi.gov.in/sites/default/files/WCD/generic_multiple_files/final_saksham_anganwadi_and_mission.pdf